A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

20 May 2026

Dr Leila Byrne
Turn The Corner Medical Clinic

Dear Dr Byrne,

Re: Ms Tyler Thorburn
DOB: 23/04/1998 Phone: 0452 499 003
Address: Unit 2 / 492 Pascoe Vale Road, STRATHMORE, VIC, 3041
Email: tylerx81@hotmail.com

Summary

- **Persistent abdominal pain and alternating bowel habit with features of pelvic floor dyssynergia.**
- **Trial of rifaximin for presumed dysbiosis and bloating.**
- **Plan for upward titration of nortriptyline if symptoms remain refractory.**
- **Pending intestinal and pelvic ultrasounds.**
- **No action required by you.**

I had the pleasure of reviewing Tyler in the clinic today. She continues to experience a similar constellation of symptoms since our last review, specifically regarding lower abdominal pain and a significantly erratic bowel habit. During recent travel to Thailand, he experienced a two-week period of severe constipation, which has since transitioned back to her baseline of opening her bowels every three to four days. She describes the stool as more formed recently but notes persistent straining, a sensation of incomplete evacuation, and the passage of mucus.

Tyler has been taking nortriptyline at a dose of 10 mg daily. Unfortunately, she has not noted any improvement in the severity or frequency of her pain. She has also reported some nausea associated with the medication, though it has not assisted with her sleep. Regarding lifestyle interventions, she has attempted to reduce her intake of artificial sweeteners and energy drinks, but her symptoms of bloating remain prominent. Due to her travel schedule, she has not yet been able to attend his pelvic floor physiotherapy appointment, though she intends to reschedule this shortly.

We discussed his clinical progress at length. Tyler remains concerned that her symptoms, including a long-standing "white tongue," are related to bacterial overgrowth or dysbiosis. While the previous stool studies were unremarkable, I have explained that these do not always provide a complete assessment of the gut microbiota.

Impression and Plan I have decided to initiate a trial of rifaximin 550 mg three times daily for two weeks. While I suspect his symptoms are multifactorial—involving visceral hypersensitivity and pelvic floor dysfunction—addressing a potential component of dysbiosis may assist with his bloating. I have directed him to a compounding pharmacy to ensure this remains cost-effective.

All appointments and enquires
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East Melbourne VIC 3002
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Fax: 03 9650 7910
Pager: 03 9387 1000
Email: office@focusgastro.com.au

Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

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Regarding the nortriptyline, we will maintain the current dose while she completes the rifaximin course. If there is no significant clinical improvement one month after finishing the antibiotics, I have advised her to increase the nortriptyline dose to 20 mg, and eventually 30 mg, provided she tolerates it. I have also suggested she trial a fibre supplement to help regulate her bowel movements and thicken the stool consistency, which may be more practical than fresh fruit given his frequent travel.

Finally, I am following up on the intestinal and pelvic ultrasounds that were requested through the public hospital, as the results have not yet reached my rooms. Once these investigations are complete and we have assessed his response to the rifaximin and medication titration, I will review her again in approximately two to three months.

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

CC: Pelvic floor physios Central Melbourne

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